



Physiotherapy & Rehabilitation Platform

Medical Report: Post-ACL Reconstruction Rehabilitation

Patient Information

Name: ahmet kalumian
Birth Date: 2025-10-28
Case ID: #24

Clinical Details

Doctor: Dr. Alarkany
Date: 2026-03-28
Description: A 28-year-old male athlete, 4 weeks post-op for ACL reconstruction using a hamstring graft. Currently experiencing mild swelling and limited range of motion (ROM) in the right knee.

Physical Measurements

Height: 175 cm
Weight: 70 kg
Blood Group: AB+

Health History

Smoker: Yes
Chronic Diseases: Yes
Activity Level: Active

Additional patient information

Injury History: Complete ACL tear occurred during a football match on January 15, 2026, due to a sudden pivoting motion. Surgery was performed on March 1, 2026.

Medications: Ibuprofen 400mg as needed for pain, Vitamin D3 supplements, and Omega-3 fish oil.

Functional Ability: Partial weight-bearing with one crutch. Knee flexion is currently at 95 degrees. Struggles with stairs and long-distance walking.

Personal Goals: Full return to competitive football within 9 months, restoration of full quad strength, and achieving 135-degree knee flexion.

Smart Clinical Analysis

Clinical Explanation:

The patient is a 28-year-old male athlete who is 4 weeks post-operative from an anterior cruciate ligament (ACL) reconstruction utilizing a hamstring graft. He presents with mild swelling and limited range of motion (ROM) in the right knee, which is common following such a surgical procedure. The patient is currently partially weight-bearing with the assistance of one crutch and has achieved a knee flexion of 95 degrees. His rehabilitation goals include a full return to competitive football within 9 months, restoration of full quadriceps strength, and achieving 135 degrees of knee flexion.

Immediate Physiotherapy Recommendations:

The focus of physiotherapy should include gentle range of motion exercises to improve knee flexion, as well as isometric quadriceps strengthening exercises. Gradual progression to weight-bearing activities should be encouraged as tolerated, while monitoring for any increase in swelling or pain. Incorporating modalities such as ice and elevation may assist in managing swelling.

Precautions:

The patient should avoid high-impact activities, sudden pivoting motions, and any exercises that cause significant pain or discomfort. Care should be taken to monitor for signs of increased swelling or instability in the knee.

Estimated Recovery Timeline:

Given the current progress and the patient's goals, a full recovery to competitive football may be anticipated within 6 to 9 months post-surgery, contingent upon adherence to rehabilitation protocols and the absence of complications.

Risk Assessment:

Potential risks include re-injury of the ACL, development of stiffness or loss of range of motion, and complications related to chronic diseases. Close monitoring of the patient's response to rehabilitation is essential.

Red Flags:

Emergency warning signs include severe swelling or pain, inability to bear weight on the affected leg, signs of infection (such as fever or increased redness), and any sudden changes in knee stability.

Test Group: Multiple Sclerosis

Test: Knee Flexion (Right)

Clinical AI Analysis

Doctor Standard/Target:
135 degrees

Clinical Interpretation:
The patient has achieved a knee flexion of 115 degrees, which is still below the target of 135 degrees. This indicates that while there is improvement, the patient has not yet reached the desired range.

Progress Evaluation:
The patient has shown a consistent improvement trend, moving from 95 degrees to 115 degrees. He remains 20 degrees away from the target, indicating the need for continued rehabilitation efforts.

Physiotherapy Focus:
Incorporate functional activities that promote knee flexion, such as step-ups and gentle squats, while ensuring proper technique to avoid undue stress on the knee.

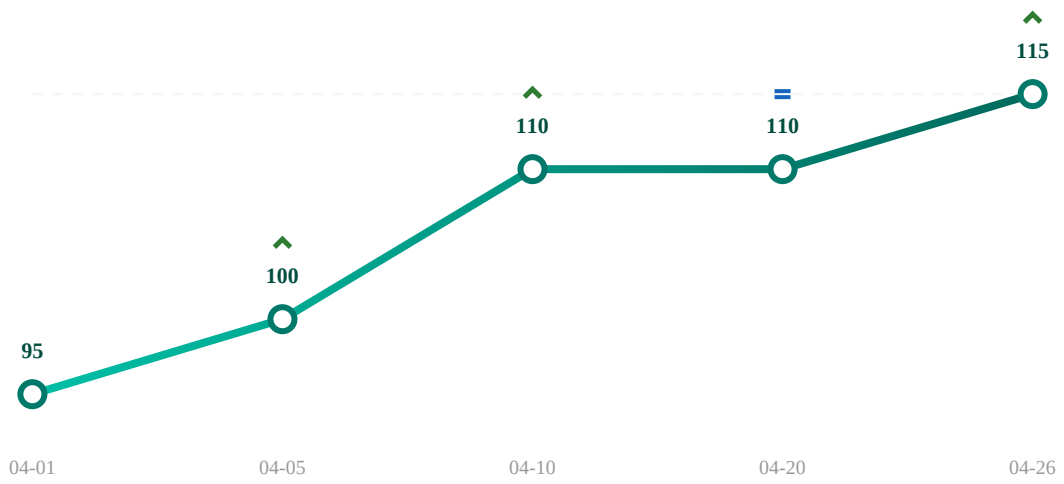
HIGHEST
115.0

LOWEST
95.0

AVERAGE
106.0

OVERALL PROGRESS

Increased by 21.1% ↑ Since first test (2026-04-01)



Date	Result	Standard
2026-04-01	95	—
2026-04-05	100	—
2026-04-10	110	—
2026-04-20	110	—
2026-04-26	115	—

Important Notice

This report is AI-assisted and intended for informational support only. It is not a medical diagnosis and does not replace professional clinical judgment. Please consult your doctor or therapist for any medical decisions.